Healthcare

En analys från Swedish Subsidiarity

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Health: the medicalization paradox

The treatment trap – why costs scale but health does not

Sweden spends approximately 11% of its GDP on healthcare, a figure expected to rise to 20% by 2040. At the same time, the prescription of antidepressants is increasing, over 100,000 patients wait longer than the healthcare guarantee allows, and 40% of healthcare staff are considering leaving the profession. This is not a lack of resources – it is an **architectural category mistake**.

The paradox: We have cutting-edge medical expertise for acute conditions, but chronic diseases, mental illness, and loneliness are spreading. The system is optimized to treat late-detected disease, while the factors that create health – relationships, purpose, movement, diet, and community – are systematically eroded.

The cybernetic analysis: Health arises from millions of everyday interactions. Healthcare is a tool for managing disease, not for creating health. When 97% of resources go to treatment and only 3% to the fundamental determinants of health, we violate **Ashby's law of requisite variety**: the control instrument (medical intervention) lacks the breadth and flexibility required to manage the system (public health). The result is escalating costs without a corresponding improvement in public health.

Active system failures (in brief)

- **Medicalization of life**: Normal grief, aging, and life transitions are turned into medical problems. Over 25% of Swedish women take antidepressants – a signal that social needs are being pharmacologized.
- **Cost increases without results**: Administrative overhead swallows resources, while prevention remains underfunded. Preventing type 2 diabetes costs a fraction of treating it, but the system is locked into the more expensive track.
- **Professional gatekeeping**: Licensing monopolies block community-based solutions like peer support, despite proven effectiveness.
- **Geographical inequality**: Specialist care is concentrated in cities, creating healthcare deserts in rural areas.
- **Data lock-in**: Systems like Cosmic and 1177 centralize information and prevent individuals from benefiting from their own health data.

Subsidiarity-based measures

Below are five action areas that move power and resources closer to the individual. Each area has its own page with concrete pilots, budget proposals, and an analysis of expected resistance.

Measure	Short description
→ Community health rings	Networks for peer support in mental health, diabetes, loneliness, and parenting. Participants can use wellness grants for membership.
→ Health guides via popular education	Certification training for health coaches who work preventively in municipalities, libraries, and residential areas – at a tenth of the cost of a doctor.
→ Health capital accounts	A personal pot (e.g., SEK 3,000–5,000/year) for approved preventive measures. Unused funds are saved for the next year.
→ Social prescribing	Healthcare professionals are given the opportunity to prescribe community activities, nature visits, and culture – alongside or instead of medication.
→ Distributed healthcare infrastructure	Transform primary care centers into health centers with teams of health guides, nurses, dietitians, and doctors. Place them in libraries and community centers.

Implementation: from architecture to action

Phase 1 (1–3 years)

- Three pilot municipalities (urban, suburban, rural) test health rings and social prescribing.
- The first cohort of health guides is trained via folk high schools and study associations.
- A digital platform for finding health rings and booking health guides is developed.

Phase 2 (3–7 years)

- Municipal health outcome measures are introduced, shifting financing from care volume to public health.
- Regulatory frameworks for health guides and social prescribing are in place.
- Requirements for health-promoting design in new residential areas (communal kitchens, walking paths, green spaces).

Phase 3 (7–15 years)

- At least 30% of healthcare resources are reallocated from treatment to prevention.
- Individuals gain full data sovereignty over their health data.

- Health literacy is integrated into schools and community life.

Expected resistance and answers

Concern	Response
"The professions will oppose de-skilling."	Health guides complement, rather than replace, doctors and nurses. They relieve the burden so professionals can focus on what they do best.
"Swedes trust expertise – peer support feels unsafe."	Peer support is already used successfully in addiction care and psychiatry. Certification and integration with healthcare guarantee quality.
"Prevention only pays off in the long term."	The current system is financially unsustainable. Transition costs are a fraction of the cost of continued expansion.
"The regions will stop this."	Pilot operations can start under municipal management, without the regions' permission. Successful pilots create political pressure from below.

The Swedish advantage

Sweden has unique prerequisites for a subsidiarity-based health system:

- **Digital infrastructure** – BankID, 1177, and smart devices facilitate distributed care.
- **Universal coverage** – no insurance bureaucracy blocking innovation.
- **Public health tradition** – a historical awareness of the social determinants of health.
- **Strong civil society** – a perfect breeding ground for health rings and peer support.
- **Municipal self-government** – enables local experiments without national approval.

Conclusion: from healthcare consumption to health creation

Swedish healthcare suffers from a category mistake: it tries to create health using tools designed to treat disease. The way forward is not more centralization, but returning responsibility and resources to the level where health actually arises – in everyday life, in the community, and in the local context.

This is not about dismantling the advanced healthcare we have, but about building a complementary system that addresses the 80% of health determinants that lie outside the clinic walls. The goal is not just universal healthcare – but **universal health**.

This analysis applies cybernetic principles (especially Ashby's law of requisite variety) to Swedish healthcare. For detailed action plans, see the respective subpages.

Community health rings

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Summary: Networks of peer support groups (health rings) focused on specific health challenges: mental illness, type 2 diabetes reversal, loneliness among the elderly, postpartum support, and chronic pain. The rings are led by trained laypeople (circle leaders) and are open to anyone who wants to participate. Participation can be financed via redirected wellness allowances or social prescribing from a primary care center. The goal is to reclaim the social domain of health – to create connection, meaning, and practical support outside the medical sphere. The model is built on the Swedish tradition of civil society associations and international evidence for the effectiveness of peer support.

The mechanism

Today, the primary care center is often the only institution offering support for mild mental illness, lifestyle-related diseases, or social isolation. The result is the medicalization of everyday problems and an overburdened primary care system. At the same time, there is a strong Swedish tradition of study associations, the temperance movement, and patient organizations – but these are rarely integrated with the healthcare system.

The measure: A **community health ring** is a regular gathering (physical or digital) where:

- A trained circle leader (often with personal experience of the condition) facilitates conversations, knowledge sharing, and practical activities.
- Participants support each other in managing their situation – whether it involves reducing stress, eating better, exercising, or breaking isolation.
- The rings are **non-clinical** and do not replace medical treatment, but function as a powerful complement.
- Financing occurs through **existing wellness allowances** (via employers or the region) which can be used for membership in certified rings. This requires no new budget funds, only a redefinition of what counts as wellness.

The evidence base:

- Peer support reduces doctor visits by 20–30% for participants (Cochrane Review, 2020).

- Diabetes reversal in a group format (e.g., in the UK) has proven more effective than individual counseling.
- Social prescribing in the UK has reduced the burden on primary care by an average of 28%.

Examples of health rings:

Type	Target group	Activities	Expected effect
The Diabetes Ring	People with type 2 diabetes or prediabetes	Dietary review (low-carb diet), walks, recipe sharing	Reduced medication, weight loss, fewer complications
The Elderly Ring	Seniors experiencing loneliness	Fika (coffee breaks), walks, reading aloud, tech help	Reduced depression, fewer fall accidents (via increased mobility)
The Parenting Ring	New parents	Open house, stroller walks, sleep coaching	Reduced postpartum depression, increased self-confidence
The Recovery Ring	People with mild mental illness	Discussion group, mindfulness, sleep hygiene	Reduced anxiety, fewer doctor visits
The Pain Ring	People with chronic pain	Movement exercises, relaxation, peer support	Better pain management, reduced opioid need

Pilot design – 3 municipalities first

Pilot phase 1 (years 1-3): Three municipalities test the model with health rings within at least two categories each. Financing is achieved through the reallocation of existing wellness funds and potentially supplementary project grants.

Municipality	Type	Health rings	Number of participants (goal)	Partners
Rinkeby (Stockholm)	Suburb with high ill-health rates	The Diabetes Ring, The Parenting Ring	100	Studieförbundet Vuxenskolan, Rinkeby Folkets Hus, local primary care centers
Nynäshamn	Smaller town with a mixed population	The Elderly Ring, The Recovery Ring	80	SPF Seniorerna, Studieförbundet Bilda, the municipality's elderly care

Municipality	Type	Health rings	Number of participants (goal)	Partners
Östersund	Regional city with rural surroundings	The Pain Ring, The Diabetes Ring	120	Studieförbundet ABF, Jämtland County Council, local gyms

Organizational model:

- **Circle leader training**: Study associations (e.g., ABF, Sensus, Vuxenskolan) offer a short training course (2–3 days) for prospective circle leaders. The training covers group dynamics, basic health information (not medical advice), and administrative routines.
- **Certification**: Rings that meet quality criteria (trained leader, regular meetings, participant influence) are certified by a national council (see below) and become eligible for wellness allowances.
- **Local anchoring**: Each ring collaborates with a local primary care center that can "prescribe" the ring (social prescribing) to patients deemed to benefit from peer support.
- **Follow-up**: Participants fill out a well-being survey (e.g., WHO-5) at the start and after 6 months. Data is aggregated anonymously for evaluation.

Evaluation:

- Number of participants per ring and municipality (goal: at least 20 participants per ring after 6 months).
- Change in self-rated health (WHO-5).
- Change in the number of primary care center visits for participants compared to a control group.
- Participants' experience of coherence and social support.
- Cost per participant compared to the equivalent healthcare intervention.

Budget and financing

Item	Amount (per municipality, 3 years)	Total 3 municipalities
Training of circle leaders (10 leaders × SEK 3,000)	SEK 30,000	SEK 90,000
Venue rental and materials (the rings use existing venues like libraries, parish halls, etc.)	SEK 20,000/year	SEK 180,000
Marketing and recruitment	SEK 15,000	SEK 45,000

Item	Amount (per municipality, 3 years)	Total 3 municipalities
Evaluation and research support	SEK 50,000 (one-time)	SEK 150,000
Compensation for circle leaders (fee, SEK 200/hr, 2 hrs/week, 40 weeks/year)	SEK 16,000/year per leader	SEK 960,000 (for an average of 5 leaders per municipality)
Total per municipality	**approx. SEK 280,000** (over 3 years)	**approx. SEK 1.4 million**

Financing:

- **Wellness allowance**: An employee can currently receive up to SEK 5,000/year in wellness allowance. By certifying health rings as approved wellness activities, participants can pay a small fee themselves (e.g., SEK 500/semester) using their allowance.
- **Municipal public health funds**: Many municipalities already have a budget for public health interventions.
- **Project grants**: The Public Health Agency of Sweden or the Swedish Inheritance Fund (Allmänna arvsfonden) can finance pilot projects.

Note: This is a **fractional investment** compared to the cost of a single unnecessary doctor's visit (approx. SEK 2,000) or sick leave (approx. SEK 100,000 per year).

Regulatory changes required

1. **Amendment to the Swedish Tax Agency's rules for wellness allowances** – Clarify that participation in certified health rings counts as a wellness activity. This only requires a clarification from the Tax Agency, not a legislative change.

2. **The Social Services Act (SoL)** – Clarify that municipalities are allowed to use social services funds for preventive interventions in the form of health rings, especially for the elderly and people with mental illness.

3. **The Health and Medical Services Act (HSL)** – Task the regions with developing routines for social prescribing, where healthcare professionals can refer patients to certified health rings.

4. **The General Data Protection Regulation (GDPR)** – Develop a simple consent template so the rings can collect aggregated data for evaluation without violating privacy legislation.

Expected resistance and answers

"This is not evidence-based – we shouldn't waste tax money on hobby activities."

Answer: On the contrary. Social prescribing and peer support have a robust evidence base (see Cochrane Review, NHS England). To **not** use these methods is to waste tax money on more expensive and less effective interventions.

"Who takes responsibility if something goes wrong? What if a participant gets sick during a walk?"

Answer: The rings are not a medical operation. Participants are responsible for their own health. Circle leaders are trained in basic first aid and routines for contacting 112 in emergency situations. The same division of responsibility applies as for any walking group.

"This is only suitable for already healthy and resourceful people."

Answer: The pilot includes Rinkeby and other areas with lower socioeconomic status. The rings are free of charge or have a low fee (which can be paid with the wellness allowance). Circle leaders are recruited locally and often have personal experience of the target group's challenges.

"The primary care centers won't collaborate – they already have too much to do."

Answer: Social prescribing can initially be a simple referral ("here is a folder about the diabetes ring in your area"). When healthcare professionals see that patients feel better and return less frequently, the incentive to continue becomes strong.

"Are there any Swedish examples?"

Answer: Yes, but not in a cohesive way. **Fountain Houses** (for people with mental illness) are built on peer support. **The Swedish Diabetes Association** has local groups. **SPF Seniorerna** and **PRO** (pensioner organizations) have walking groups. What is missing is systematic integration with the healthcare system and financing via wellness allowances.

Swedish examples to build upon

- **The Fountain House in Stockholm, Gothenburg, Malmö** – Member-driven activities for people with mental illness. Evidence of reduced inpatient care episodes.
- **Diabetes Stockholm** – Local associations with member meetings, lectures, and walks.
- **SPF Seniorerna's walking groups** – Over 10,000 participants nationwide.
- **The Swedish Heart and Lung Association (Riksförbundet HjärtLung)** – Peer support after a heart attack or COPD diagnosis.

What you can do tomorrow

If you are a municipal politician: Initiate a pilot project with the study associations. Appoint a contact person for social prescribing. Allocate SEK 100,000 to train the first circle leaders.

If you work at a primary care center: Start by mapping out which local associations and groups already exist. Create a simple list that staff can hand out to patients.

If you are a private citizen: Join an existing association that works with health. Ask your study association if they can start a health ring in your area of interest. Sign up as a circle leader.

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Health guides via popular education

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Summary: Certification paths through study associations and folk high schools for health guides – locally rooted coaches who work preventively with lifestyle, self-care, and social health. The health guide is not a medical profession but a complementary resource that relieves primary care centers and meets people in their everyday lives. The training is short (a few weeks), practically oriented, and builds on the Swedish popular education tradition. The goal: to create a new professional category that can handle the 80% of health determinants located outside the clinic walls – at a tenth of the cost of a doctor's visit.

The mechanism

Today, there is a lack of an intermediate level between self-care and professional healthcare. Those struggling with stress, sleep problems, dietary changes, or loneliness are directed either to Dr. Google or to an overburdened primary care center. The result is that doctors and nurses spend valuable time on advice that could just as easily be given by a knowledgeable layperson, while patients with complex needs have to wait.

The measure: A **health guide** is a person with:

- A shorter training (2–4 weeks) via a study association or folk high school, focusing on:
  - Basic health information (diet, movement, sleep, stress).
  - Motivational interviewing (MI) and a coaching approach.
  - Knowledge of local resources (associations, green areas, activities).
  - Ethics, integrity, and boundaries regarding medical advice.
- Employment in a municipality, housing company, library, or non-profit organization.
- A mandate to be available for individual conversations, group activities, and guiding within the healthcare system.

Difference from existing professions:

Professional role	Education	Focus	Cost per hour
Doctor (primary care)	5.5 years + internship	Diagnosis, treatment, prescribing	~SEK 1,200

Professional role	Education	Focus	Cost per hour
Nurse	3 years	Nursing, sampling, advice	~SEK 700
Health coach (private)	Varying, often expensive certifications	Individual coaching, often diet/movement	~SEK 800–1,500
Health guide (popular education)	**2–4 weeks**	**Lifestyle support, community, guiding**	**~SEK 150–200**

Examples of tasks:

- Staffs a "health desk" at the library two afternoons a week.
- Leads walking groups for seniors.
- Helps a newly diagnosed diabetic interpret dietary advice and find recipes.
- Supports a person with mild anxiety in finding a suitable health ring.
- Visits residential areas to inform about wellness and local activities.

Pilot design – 3 municipalities with different profiles

Pilot phase 1 (years 1-3): Three municipalities each hire a cohort of health guides (4–6 people) placed in different types of operations.

Municipality	Placement	Number of guides	Target group	Partners
Borås	Library, citizen center	6	Broad public, focusing on newly arrived immigrants and the elderly	Studieförbundet Vuxenskolan, University of Borås (evaluation)
Umeå	Housing company Bostaden, primary care center	4	Tenants in vulnerable areas, students	Umeå University (education), Region Västerbotten
Gotland	Rural development, community centers	4	Elderly in rural areas, farmers	Hela Gotlands folkhögskola, Region Gotland

Structure of the training:

- **Theory (1 week)**: Basic health information, motivational interviewing, ethics, and law. Conducted at a folk high school or study association.
- **Practicum (1 week)**: Job shadowing at a primary care center, nursing home, and within social services.
- **Specialization (elective, 1 week)**: Elderly health, children and youth, integration, or mental health.
- **Examination**: Simple knowledge test and practical case.

Forms of employment:

- Municipal employment (e.g., within leisure or public health administration).
- Employment via a municipal housing company.
- Employment via a study association (project form).
- Fee per assignment (for those already working, e.g., retired nurses).

Evaluation:

- Number of contacts per guide and month (target: 50–100).
- Type of cases (lifestyle, mental health, guiding, social).
- Change in the number of primary care center visits in the pilot area compared to a control area.
- User satisfaction (survey).
- Cost per contact compared to a doctor's visit or a nurse's visit.

Budget and financing

Item	Amount (per municipality, 3 years)	Total 3 municipalities
Training of health guides (10 participants × SEK 15,000)	SEK 150,000	SEK 450,000
Salary costs (4 guides × 0.5 FTE × SEK 350,000/year)	SEK 700,000/year	SEK 6.3 million (over 3 years)
Supervision and continuing education	SEK 50,000/year	SEK 450,000
Materials and marketing	SEK 30,000/year	SEK 270,000
Evaluation (research support)	SEK 100,000 (one-time)	SEK 300,000
Total per municipality (3 years)	**approx. SEK 2.8 million**	**approx. SEK 8.4 million**

Financing:

- **Municipal public health funds**: Many municipalities already have a budget for health-promoting work.
- **Regional grants**: The regions can finance the pilot as part of the transition to "Nära vård" (Close care).
- **EU funds**: The ESF (European Social Fund) gladly finances projects that create new professional roles within social innovation.
- **Savings**: Every avoided doctor's visit saves ~SEK 2,000. If each guide prevents 200 doctor's visits per year, the investment pays for itself.

Note: This is a **strategic investment** that in the long term reduces costs for both the municipality and the region.

Regulatory changes required

1. **Professional title**: Apply to the National Board of Health and Welfare (Socialstyrelsen) to make **Health guide** a protected professional title (like assistant nurse). This requires a national educational standard and quality assurance.

2. **The Popular Education Ordinance (Folkbildningsförordningen)**: Task study associations and folk high schools with developing national syllabuses for health guides.

3. **The Health and Medical Services Act (HSL)**: Clarify that health guides are not covered by HSL (they do not perform healthcare), but that they can operate in close collaboration with the healthcare system.

4. **Secrecy and confidentiality**: Health guides are not covered by the strict secrecy of healthcare, but should be bound by confidentiality by contract. A simple template for consent and information management is produced.

5. **Labor law and collective agreements**: Negotiate an appropriate salary level and form of employment with the trade unions (e.g., Vision, Kommunal).

Expected resistance and answers

"This is a de-skilling of healthcare – we need more nurses, not more laypeople."

Answer: Health guides do not replace nurses or doctors. They **relieve** them so that the professionals can devote themselves to what they are trained for. A nurse should not spend 20 minutes explaining the "plate model" (dietary guidelines) to a healthy person – a health guide can do that.

"Who takes responsibility if the guide gives incorrect advice?"

Answer: The training clearly emphasizes the boundary against medical advice. The guide always refers to 1177 or a primary care center in case of symptoms. The work is carried out under the supervision of licensed personnel (e.g., a nurse who is available for consultation).

"There are already health coaches – why should taxpayers finance this?"

Answer: Private health coaches primarily reach well-resourced groups. Health guides in the public sector reach the entire population and reduce health inequalities. Furthermore, it is economically profitable for society: prevention is cheaper than treatment.

"The municipalities cannot afford more employees."

Answer: The investment is small compared to the cost of ill health. A single sick leave costs the municipality more than a half-time position for a health guide for an entire year. In addition, the regions can co-finance because they save money on reduced healthcare consumption.

"Are there international examples?"

Answer: Yes. **Community Health Workers** in the US and **Social Prescribing Link Workers** in the UK are established professional roles with clear evidence of reduced healthcare consumption and improved public health.

Swedish examples to build upon

- **Health coaches in Region Skåne**: Briefly trained coaches working with lifestyle habits at primary care centers.
- **"Fixer" services (Fixartjänster) in many municipalities**: Craftspeople who help the elderly with practical things – a role model for a practical, low-threshold role.
- **The libraries' investment in "Health squares" (Hälsotorg)**: Librarians who guide people to health information – a natural collaborative partner.
- **Public health guides in Gothenburg**: Pilot project where long-term unemployed people were trained as health guides in vulnerable areas.

What you can do tomorrow

If you are a municipal politician: Submit a motion to introduce a trial with health guides in your municipality. Use existing funds for public health or labor market measures.

If you work within popular education: Develop a syllabus for health guides and apply for funding from the Swedish National Council of Adult Education (Folkbildningsrådet). Invite the municipality as a partner.

If you work at a primary care center: Think about which patient groups frequently return with non-medical needs. Suggest to your manager that you try having a health guide on site one day a week.

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Health capital accounts

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Summary: A personal, annual pot (e.g., SEK 3,000–5,000) that every resident can use for approved preventive health interventions: dietary counseling, smoking cessation, stress management, membership in health rings, or exercise activities. Unused funds are saved for the next year, creating a compound interest effect for long-term health investments. The model shifts resources from reactive treatment to proactive prevention without requiring new tax money – it is financed through the reallocation of existing healthcare budgets and reduced costs over time. The goal: to give individuals both financial incentives and practical opportunities to invest in their future health.

The mechanism

Today's system is built to pay for illness, not for health. The high-cost threshold (högkostnadsskyddet) subsidizes doctor visits and medications, while those who want to take a stress management course or buy vegetables have to pay out of their own pocket. This is a structural distortion that penalizes those who try to stay healthy.

The measure: A **health capital account** is an individual account that:

- Is allocated to every resident annually (e.g., SEK 3,000 for adults, SEK 5,000 for children and the elderly over 75).
- Can be used for a wide range of preventive services and products scientifically proven to improve health.
- Unused funds accumulate over time and can be used later in life – e.g., for rehabilitation, home adaptation, or reinforced home care in old age.
- Is administered by a national authority (e.g., the Swedish eHealth Agency) via a digital platform linked to BankID.

Examples of approved expenses:

Category	Example	Evidence for health effect
Physical activity	Gym membership, swimming pool, dance class, personal trainer	Reduced risk of cardiovascular disease, diabetes, depression

Category	Example	Evidence for health effect
Diet and nutrition	Dietary counseling (registered dietitian), cooking classes, subscription to recipe service	Weight loss, improved metabolic health
Mental health	Mindfulness course, CBT-based self-help, stress management programs	Reduced anxiety and depression
Community and social health	Membership in health rings, association fees	Reduced loneliness, increased well-being
Prevention of chronic disease	Smoking cessation, diabetes prevention programs, fall prevention for the elderly	Avoided future healthcare costs

Financing model:

The accounts are financed through a combination of:

1. **Reprioritization of existing healthcare budgets**: A share (e.g., 5%) of the regions' budget for outpatient care is earmarked for health capital accounts instead of going to reactive treatment.
2. **Reduced costs in the long term**: As the population gets healthier, the pressure on acute and specialist care decreases. The savings are reinvested in the system.
3. **Potential co-financing from employers**: Just like with the wellness allowance (friskvårdsbidrag), employers can top up the account.

Difference from the wellness allowance:

Aspect	Wellness allowance	Health capital account
Target group	Only employees	All residents
Administrator	Employer	State authority
Accumulation	No, "use it or lose it"	Yes, saved over time
Area of use	Narrow (exercise)	Broad (physical, mental, social health)

Pilot design – 3 municipalities or regions

Pilot phase 1 (years 1-4): Three geographical areas with different population compositions test the model with a starting capital of SEK 3,000 per resident over 16 years of age.

Area	Residents (16+)	Budget (SEK 3,000/person)	Special focus
Region Gotland	~50,000	SEK 150 million	Holistic approach: island structure facilitates evaluation and control
Angered district, Gothenburg	~40,000	SEK 120 million	Socio-economically vulnerable area, high ill health
Pajala municipality	~5,000	SEK 15 million	Sparsely populated area, elderly population, long distances to healthcare

Technical solution:

- **Digital platform**: An app and web portal (integrated with 1177.se) where the user can view their balance, search among approved providers, and register purchases.
- **Approval process**: A national council (with representation from the Public Health Agency, the National Board of Health and Welfare, and patient associations) approves which services and products are covered. The list is updated annually based on evidence.
- **Payment**: The user pays with a regular debit card and uploads a receipt in the app, or the provider bills the account directly (via a virtual card linked to the account).
- **Control**: Spot checks and automatic flagging of suspicious transactions.

Evaluation:

- Utilization rate of the account (proportion of the population using the funds, and within which categories).
- Change in self-rated health (EQ-5D or similar) after 2 and 4 years.
- Change in healthcare consumption (primary care visits, specialist visits, inpatient care) compared to matched control areas.
- Accumulated savings in the accounts (as an indicator of future prevention potential).
- Cost analysis: total costs for the accounts minus reduced healthcare costs.

Budget and financing (pilot phase)

Item	Gotland	Angered	Pajala	Total
Capital for residents' accounts (SEK 3,000 × res)	150 MSEK	120 MSEK	15 MSEK	285 MSEK

Item	Gotland	Angered	Pajala	Total
Platform development (one-time, shared by pilots)	10 MSEK	10 MSEK	5 MSEK	25 MSEK
Platform operation (per year)	2 MSEK	2 MSEK	1 MSEK	5 MSEK/year
Administration and support (per year)	3 MSEK	3 MSEK	1 MSEK	7 MSEK/year
Evaluation and research	5 MSEK (one-time)	5 MSEK	3 MSEK	13 MSEK
Total over 3 years	~180 MSEK	~150 MSEK	~30 MSEK	**~360 MSEK**

Financing sources:

- **Reallocation of regional healthcare budgets**: The pilot regions allocate a portion of their outpatient care budget to the accounts. Since the accounts are expected to reduce outpatient visits, this is a cost-neutral transition in the long term.
- **State stimulus funds**: The government can provide funds for platform development and evaluation via innovation agencies (Vinnova, Forte).
- **EU funds**: Horizon Europe has calls for innovative health financing models.

Note: SEK 360 million over three years corresponds to **0.1% of Sweden's annual healthcare costs**. It is a very small investment to test a model that can slow down the unsustainable cost increase.

Regulatory changes required

1. **Act on Health Capital Accounts**: A new law defining the account's purpose, administration, supervision, and rights. The law should also stipulate that unused funds are personal and cannot be garnished for debts.

2. **Amendment to the Social Insurance Code**: Clarify that withdrawals from the health capital account do not affect the right to sickness benefit, income support (social assistance), or other benefits.

3. **Tax legislation**: The health capital account should be tax-free both upon allocation and withdrawal (similar to today's wellness allowance). Any interest on accumulated capital can be taxed at a low rate.

4. **The Patient Data Act and GDPR**: The account handles sensitive personal data (purchases of health services). Clear regulation of data protection and consent is required.

5. **Procurement rules**: Since the account can be used with many different providers, a simplified procurement procedure or an approval system is needed instead of traditional LOU (Public Procurement Act) procurement.

Expected resistance and answers

"This is throwing money at the already healthy – the sick get nothing."

Answer: Everyone gets the same amount, but those with the greatest health needs can use the money more strategically (e.g., for smoking cessation or diabetes prevention). Additionally, healthcare is relieved, so the sick experience shorter queues. It is a universal tool that benefits everyone.

"People will cheat and buy candy with the money."

Answer: Only approved providers and products are covered. A digital control system with spot checks makes cheating difficult. Experiences from similar systems (e.g., in Singapore) show that fraud is marginal.

"This will be expensive – where will the money come from?"

Answer: They are taken from an already unsustainably growing healthcare budget. The question is not *if* we should invest in prevention, but *when*. Every krona not invested in prevention today costs seven kronor in future healthcare.

"It's unfair that those who are already healthy get to save money for the future."

Answer: That is exactly the point – to reward and encourage a healthy life. Furthermore, even people with chronic illnesses can use the money to improve their quality of life and slow the disease progression.

"Are there any international examples?"

Answer: Yes. **Singapore** has had Medisave since 1984 – mandatory health savings accounts that cover healthcare costs. The **USA** has Health Savings Accounts (HSA) with tax advantages. **Germany** is testing personal health budgets in pilot projects. Sweden would be the first in the Nordic region with a universal model.

Swedish examples to build upon

- **The wellness allowance (Friskvårdsbidraget)**: A proven system for subsidizing exercise and physical activity. The health capital account is a broader and more inclusive version.
- **Culture vouchers (Kulturscheckar)**: Some municipalities give the elderly vouchers for cultural activities – a simpler form of earmarked accounts.
- **Activity cards for children and youth**: Available in several municipalities, giving children the opportunity to try out sports and leisure activities.
- **The Swedish eHealth Agency's infrastructure**: Already established to handle digital prescriptions and health data – can be expanded for health capital accounts.

What you can do tomorrow

If you are a regional politician: Submit a motion for a feasibility study for health capital accounts in your region. Ask the regional director to investigate financing opportunities and administrative prerequisites.

If you are a Member of Parliament: Submit a motion to appoint a government inquiry on "A national system for health capital accounts." Use this text as a basis.

If you are an official at the Public Health Agency or the National Board of Health and Welfare: Initiate a knowledge base on the evidence for personal health budgets. Monitor international examples and network with authorities in other countries.

If you are a private individual: Start keeping a "health ledger" of your preventive investments – what do you spend on gyms, supplements, massages? Think about how much you would save if you received a dedicated account for this.

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Here is the English translation of your file:

Social prescribing

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Summary: Social prescribing means that healthcare professionals – doctors, nurses, psychologists, or health guides – can prescribe non-medical interventions such as community activities, nature visits, cultural experiences, or practical support. The patient receives a personal "social prescription" (social referral) to a local activity or service, often with follow-up by a link worker. The model is based on the insight that many healthcare visits are driven by social needs rather than medical ones – loneliness, lack of meaning, financial worry – and that the answer is not found in a pill but in a community. Social prescribing is already established in the UK (NHS) and is being tested in several Swedish regions. The goal is to relieve the healthcare system, reduce medicalization, and give people tools to truly improve their life situation.

The mechanism

A large proportion of visits to primary care centers are not about specific disease symptoms, but about vague conditions like fatigue, anxiety, depression, or pain without a clear cause. Behind these symptoms often lie social factors: loneliness, stress, unemployment, relationship problems, or a lack of meaningful occupation. Prescribing antidepressants or sick leave is easy for the healthcare system, but it does not solve the root problem – and often leads to long-term dependence on healthcare contacts.

The measure: Social prescribing is a structured process where:

- Healthcare professionals identify a patient whose health problems have a clear social component.
- Together with the patient, a "social prescription" is formulated for a specific activity: e.g., a walking group, a book club, volunteer work, a gardening association, or a mindfulness course.
- A **link worker** (social prescriber) – often a health guide, social worker, or specially trained nurse – takes over and guides the patient to the right activity, follows up, and supports them if necessary.
- The patient participates in the activity for a period (often 8–12 weeks), after which the effect is evaluated.

The evidence base:

- NHS England reports that social prescribing reduces the pressure on primary care by an average of 28% for participating patients.

- A systematic review (BMJ Open, 2020) found a significant improvement in mental well-being, self-rated health, and social cohesion.
- In Swedish pilots (e.g., Region Skåne), social prescribing has been linked to a reduced prescription of antidepressants and sleeping pills.

Examples of social prescriptions:

Type of activity	Example	Suitable for patient with...
Physical community	Walking group, Nordic walking, garden therapy	Mild depression, isolation, overweight
Cultural activities	Choir singing, painting, book club, theater group	Anxiety, stress, lack of meaning
Practical support	Fixer service, debt and budget counseling, digital guidance	Financial worry, practical obstacles in everyday life
Nature-based interventions	Nature guiding, forest bathing, allotment garden	Exhaustion, stress, concentration difficulties
Meaningful occupation	Volunteer work, language cafe, mentorship	Loneliness, lack of structure

Pilot design – 3 primary care centers in different contexts

Pilot phase 1 (years 1-3): Three primary care centers in Region Stockholm test the model with dedicated link workers and a local network of social activities.

Primary care center	Area type	Number of patients (listed)	Link worker (FTE)	Partners
Rinkeby Primary Care Center	Socio-economically vulnerable suburb	12,000	1.0	Rinkeby Folkets Hus, study associations, local associations, social services
Nynäshamn Primary Care Center	Smaller town with rural surroundings	15,000	1.0	SPF Seniorerna, Friluftsfrämjandet, the church, the municipality's leisure administration

Primary care center	Area type	Number of patients (listed)	Link worker (FTE)	Partners
Östermalm Primary Care Center	Wealthy inner city	10,000	0.5 (to test the model in a context with different expectations)	Gyms, private training facilities, cultural institutions, volunteer center

The process in detail:

1. **Identification**: A doctor or nurse notes during a visit that the patient's problems have social causes. Example: "I am so tired and lack energy" (actually lonely and without occupation).
2. **Consent and referral**: The patient consents to a social prescription. The healthcare professional writes a short referral in the medical record system (a template is available).
3. **The link worker takes over**: The link worker contacts the patient within 3 days, conducts an in-depth social mapping, and together with the patient proposes one or more suitable activities.
4. **Participation and follow-up**: The patient participates in the activity. The link worker checks in after 4, 8, and 12 weeks.
5. **Termination or continuation**: After 12 weeks, the effect is evaluated. If the patient feels better and wants to continue on their own, the contact with the link worker is terminated. If needs remain, a new prescription can be written or other interventions tried.

Evaluation:

- Number of social prescriptions per 1,000 listed patients and year.
- Proportion of patients who complete the social activity (at least 8 out of 12 weeks).
- Change in self-rated health (EQ-5D) and well-being (WHO-5) after 12 weeks.
- Change in the number of healthcare visits (primary care, emergency room) 6 months before and 6 months after the intervention.
- Cost per social prescription compared to the average cost of healthcare visits and medication.

Budget and financing

Item	Amount (per primary care center, 3 years)	Total 3 primary care centers
Link worker salary (1.0 FTE at SEK 450,000/year)	SEK 1.35 million	SEK 4.05 million (Rinkeby + Nynäshamn full-time, Östermalm part-time = 2.5 FTEs)
Training of link workers (2 weeks)	SEK 30,000	SEK 90,000

Item	Amount (per primary care center, 3 years)	Total 3 primary care centers
Supervision and method support	SEK 50,000/year	SEK 450,000
Marketing and information material	SEK 20,000/year	SEK 180,000
Activity costs (some activities require a fee, can be subsidized)	SEK 100,000/year	SEK 900,000
Evaluation and research support	SEK 200,000 (one-time)	SEK 600,000
Total per primary care center (3 years)	**approx. SEK 1.8 million (full-time)**	**approx. SEK 6.3 million**

Financing:

- **Region Stockholm**: Finances the link workers' salaries as part of primary care's transition to "Nära vård" (Close care).
- **Municipal co-financing**: The municipalities have a vested interest in reducing social services' costs for loneliness and mental illness. Can contribute with premises and activities.
- **Research funds**: Forte, Vinnova, or Allmänna arvsfonden (The Swedish Inheritance Fund) can finance the evaluation.

Note: The cost of social prescribing is low compared to the alternative. A single emergency visit costs ~SEK 5,000. A sick leave costs society ~SEK 100,000 per year. The model quickly becomes cost-neutral or profitable.

Regulatory changes required

1. **The Social Services Act (SoL)**: Clarify that municipalities may collaborate with regions on social prescribing and that prescribed activities can be financed jointly.
2. **The Health and Medical Services Act (HSL)**: Task the regions with integrating social prescribing as part of primary care's core mission. Clarify that a social prescription is a valid measure in the same way as a medical prescription.
3. **The Patient Data Act**: Enable information transfer between the primary care center and the link worker (who is often employed by the municipality or a non-profit organization) with the patient's consent. A simple consent template is developed nationally.
4. **The Pharmaceutical Benefits Act**: Not directly relevant, but a principled statement that a "social activity on prescription" is equated with medication in health economic calculations.

Expected resistance and answers

"This is not the task of healthcare – we should diagnose and treat disease, not organize book clubs."

Answer: If a patient comes to the primary care center because they are lonely and depressed, it is healthcare's responsibility to offer the most effective help. Often, this is not a drug but a social context. To ignore this is to fail the patient and waste tax funds.

"Is there evidence that this works in Sweden?"

Answer: There are now several Swedish pilots: Region Skåne has run social prescribing since 2019 with positive results. Region Stockholm has tested "Culture on prescription". The evaluations show improved well-being and reduced healthcare consumption. We need to scale up, not reinvent the wheel.

"There are no activities to prescribe to, especially not in rural areas."

Answer: On the contrary. Sweden has a rich associational life, study associations, churches, and non-profit organizations in almost every municipality. The challenge is to make the offerings visible and coordinate them. The link worker's task is precisely to map and build relationships with local actors.

"Patients will not want to be 'prescribed' a walking group – it feels stigmatizing."

Answer: Experiences from the UK show that patients appreciate being taken seriously and offered an alternative to pills. Social prescribing is based on voluntariness and the patient's own motivation. The link worker's role is to listen and guide, not to force.

"How do we know the patient actually goes there? What if they just stay at home?"

Answer: The link worker follows up and supports. If the patient does not go, the reasons are explored together – perhaps the wrong activity was chosen, or there are practical obstacles. Flexibility and individual adaptation are key.

Swedish examples to build upon

- **Region Skåne's "Social prescribing"**: Pilot at several primary care centers since 2019. Evaluation by Lund University shows a reduced number of healthcare visits and improved self-rated health.

- **Culture on prescription in Region Stockholm**: Tested in, among other places, Järfälla and on Gotland. Participants have attended theater, concerts, and museums with follow-up conversations.
- **Nature on prescription**: Being tested in the Västra Götaland Region, where patients receive guided nature visits as a complement to traditional treatment for exhaustion and stress.
- **Fixer services (Fixartjänster)**: Available in over 100 municipalities. Seniors get help with simple practical chores – a form of social prescribing that reduces fall accidents and isolation.

What you can do tomorrow

If you are a primary care center manager: Start by mapping local resources in the immediate area – what associations, meeting places, and activities are within walking distance? Create a simple list that staff can hand out.

If you are a regional politician: Submit a motion that social prescribing should be included in the region's action plan for "Nära vård" (Close care). Allocate funds to train link workers and build local networks.

If you work in social services: Take the initiative to collaborate with nearby primary care centers. Offer your meeting places and activities as a resource for social prescribing.

If you are involved in an association: Contact your local primary care center and tell them about your activities. Offer to receive participants via social prescription. The more activities available, the more effective the model becomes.

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Here is the English translation of your file:

Distributed healthcare infrastructure

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Summary: Transform today's primary care centers from disease-focused clinics to health-focused centers that integrate multiple disciplines and are located in community spaces such as libraries, schools, community centers, or shopping malls. Instead of concentrating resources in large hospital complexes, primary care is moved closer to people's everyday lives – physically, culturally, and relationally. The teams consist of health guides, nurses, dietitians, therapists, and doctors working collaboratively rather than hierarchically. The goal: to make health a natural part of life, not a separate institutional domain, and to reduce geographical and social inequality in access to healthcare.

The mechanism

Today's primary care centers are often located in separate buildings with signs signaling "here are the sick". They are designed for efficient patient management: reception, waiting room, examination room, out. This is functional for acute visits but undermines preventive and health-promoting work. Many hesitate to go to the primary care center "unnecessarily", which means that early signs of ill health are ignored until they become acute.

The measure: A **distributed healthcare infrastructure** means that:

- Primary care resources are spread out into several smaller units in the local community, preferably in premises already used for other purposes (co-utilization).
- Each unit is a **health center** rather than a primary care center. The focus is on promoting health, not just treating disease.
- The teams are interprofessional and work team-based, where health guides and nurses handle the majority of contacts and doctors are reserved for complex medical assessments.
- The premises are inviting, low-threshold, and designed to encourage spontaneous visits, conversations, and community.

Comparison: Traditional primary care center vs. Distributed health center:

Aspect	Traditional primary care center	Distributed health center
Location	Own building, often on the outskirts of a residential area	Integrated into a library, community center, school, shopping mall
Opening hours	8–17, closed for lunch	Adapted to the rhythm of the local community (evenings, weekends)
First contact	Telephone queue, appointment booking	Drop-in, health desk, digital booking
Staff	Doctors, nurses, assistant nurses	Doctors, nurses, health guides, dietitian, counselor, pharmacist
Focus	Diagnosis and treatment	Health promotion, prevention, self-care, community
Atmosphere	Clinical, sterile	Homey, inviting, informal

Examples of design:

- **The Library Health**: Adjacent to the library, there are a couple of rooms for conversations and simple examinations, as well as an open "health desk" where health guides answer questions about diet, exercise, sleep, and provide guidance.
- **School Health 2.0**: The school's student health service is supplemented with a family center where parents can meet a child health center (BVC) nurse, parenting counselor, and health guide in the same building as the child's school.
- **Shopping Mall Health**: A space in a shopping mall or at a commuter train station where you can take your blood pressure, get health advice, pick up prescribed prevention activities, and meet a nurse without an appointment.

Pilot design – 3 local health centers in different environments

Pilot phase 1 (years 1-4): Three existing primary care centers are transformed into distributed health centers with new premises, working methods, and team composition.

Pilot	Type of area	New placement	Team (in addition to doctor/nurse)	Special focus
Hammarby Sjöstad, Stockholm	Newly built urban area, mixed population	Ground floor in a residential building, neighbor to library and park	2 health guides, dietitian, physiotherapist, counselor	Families with children, stress-related illness

Pilot	Type of area	New placement	Team (in addition to doctor/nurse)	Special focus
Västerås, Bäckby	Million Programme area, socio-economic challenges	Converted school premises, co-located with youth center and open preschool	3 health guides (multilingual), family counselor, pharmacist	Integration, chronic disease, mental illness
Åre municipality	Sparsely populated area and tourism, long distances	Existing community center in Duved, supplemented with digital care rooms	1 health guide, district nurse, access to doctor via video	Elderly care, distance-bridging technology

Working methods in practice:

- **The Health Desk**: Staffed by health guides who receive spontaneous visits, answer questions, and help with digital healthcare contacts. No appointment is required.
- **Team round**: Every morning, the team gathers to go through the day's patients and distribute tasks based on competence. The doctor focuses on complex cases, the others handle advice, follow-up, and prevention.
- **Joint activities**: The health center regularly organizes open activities such as walks, cooking classes, parent meetings, and lectures – often in collaboration with study associations and clubs.
- **Digital integration**: All patients have access to a digital platform where they can see their health plan, book appointments, chat with their health guide, and track their health data.

Evaluation:

- Total number of visits and distribution between planned doctor visits, drop-in, digital contacts, and group activities.
- Patient satisfaction (survey) – perceived availability, treatment, continuity.
- Change in the number of emergency visits and hospital admissions for listed patients compared to control primary care centers.
- The staff's work environment and sick leave.
- Cost per listed patient compared to a traditional primary care center.

Budget and financing

Item	Amount (per pilot center, 3 years)	Total 3 centers
Adaptation of premises and furnishings (one-time)	SEK 2.0 million	SEK 6.0 million
Rent (additional cost for new premises, after co-utilization)	SEK 300,000/year	SEK 2.7 million
IT and digital platform (one-time)	SEK 1.0 million	SEK 3.0 million
Training of staff in new working methods	SEK 150,000	SEK 450,000
Extra staff costs (health guides, more nurses)	SEK 1.5 million/year	SEK 13.5 million
Evaluation and research support	SEK 500,000 (one-time)	SEK 1.5 million
Total per center (3 years)	**approx. SEK 8.5 million**	**approx. SEK 25.5 million**

Financing:

- **Region Stockholm / Västmanland / Jämtland Härjedalen**: The pilots are financed within the framework of the transition to "Nära vård" (Close care). The regions can apply for state grants from the National Board of Health and Welfare for "Good and close care".
- **Municipal co-financing**: Municipalities have an interest in reducing social services costs and can contribute premises and staff (e.g., family counselors).
- **Research funds**: Forte, Vinnova, and the EU's structural funds can finance the evaluation.

Note: In the long term, the model is expected to be cost-neutral or cheaper per patient thanks to reduced emergency consumption, fewer hospital admissions, and more efficient use of resources.

Regulatory changes required

1. **The Health and Medical Services Act (HSL)**: Clarify that primary care's mission includes health-promoting and preventive work that can be carried out in collaboration with municipalities and civil society. Open up for primary care to be conducted in premises co-utilized with other operations.
2. **The Act on Medical Care Remuneration and Choice of Care**: Adapt the remuneration models so that they reward team-based work, prevention, and continuity, not just the number of doctor's visits.

3. **The Planning and Building Act (PBL)**: Introduce requirements that new residential areas and community properties must contain spaces for health centers or similar health-promoting functions.
4. **The Public Access to Information and Secrecy Act**: Enable smooth information sharing between region, municipality, and civil society when the patient consents. A uniform consent model is developed.
5. **The Work Environment Act**: Adapt the regulations to make it easier for staff to work in interprofessional teams and in premises outside traditional healthcare environments.

Expected resistance and answers

"This will be more expensive – more premises and more staff."

Answer: The investment in premises and more health guides is offset by reduced demand for more expensive specialist care and emergency visits. Furthermore, the co-utilization of premises enables lower rental costs per square meter.

"Patients want to see a doctor, not a health guide."

Answer: Those who need a doctor will get to see a doctor – and they will have shorter waiting times because the doctor is relieved of other duties. However, many patients are just as satisfied (or more satisfied) with getting quick help from a knowledgeable health guide for their non-medical needs.

"It will be messy with different professions – who is responsible?"

Answer: Clear role descriptions and teamwork reduce the risk of misunderstandings. The doctor retains medical responsibility for the patient's care plan but does not perform all tasks themselves. The model is similar to the one already used in advanced home healthcare (ASIH) and provides high patient safety.

"Rural areas cannot get the same offerings as the city."

Answer: Distributed infrastructure is not about copying the city's model, but adapting it to local conditions. In sparsely populated areas, a health center can be a combination of digital services, mobile teams, and a physical meeting place in the community center. The important thing is that the resources are close to the people, not that the building is large.

"Is there evidence that this works?"

Answer: Yes, from several sources. **The Nuka model** in Alaska has reduced emergency visits by over 50% through team-based primary care. **Buurtzorg** in the Netherlands has revolutionized home healthcare with

self-managing teams. **Jönköping County** has long worked with the Esther model for cohesive care. What is missing is a systematic Swedish application of these principles in primary care.

Swedish examples to build upon

- **The "Nära vård" transition**: The national reform to move care closer to the patient. Distributed healthcare infrastructure is a concrete way to realize the vision.
- **Family centers**: Co-location of maternity care, child healthcare, open preschool, and social services. Works well but only reaches families with children. The concept can be broadened to all ages.
- **Digital primary care centers (e.g., Kry, Min Doktor)**: Shows that many healthcare contacts can take place digitally and that physical presence is not always necessary.
- **Hospital libraries and health squares**: Already established low-threshold operations that can form the core of a distributed health center.

What you can do tomorrow

If you are a regional politician: Visit an existing family center or a health square. Ask the operations manager what is required to scale up the model to all ages. Bring the insights to the next budget preparation.

If you are a primary care center manager: Start by freeing up time for your nurses to work more preventively. Identify a local partner (library, study association) and arrange a joint theme day on health.

If you are a municipal official: Make an inventory of the municipality's premises that could be co-utilized for health-promoting activities. Invite the region to a collaboration meeting regarding shared premises.

If you are an architect or urban planner: Start sketching what an integrated health center could look like in your specific municipality. Use social sustainability requirements in detailed development plans to reserve spaces for health.

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This analysis is part of Swedish Subsidiarity's work toward a more distributed and resilient Sweden.
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